



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمه محمود احمد غانم
المحافظة : الجيزة
الحى / المدينة : الهرم
رقم البطاقة : 25810071700841 رقم التليفون : 01002093385
العنوان : ---
العمر (تاريخ الميلاد) : 07-10-1958 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمه محمود احمد غانم National ID: 25810071700841	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



قائمه
محمود احمد غانم
٢٠ ش - الامام مالك - فيصل
الهرم - الجيزة

٢٥٨١٠٠٧١٧٠٠٨٤١

LN7830422

٢٠٢٥/١٢ ٢٥٨١٠٠٧١٧٠٠٨٤١



ربة منزل
أنثى
مسلمة

البطاقة سارية حتى ٢٠٢٢/١٢/٢٢

MDT FORM

6 / 7 / 2025 G

Name	فايزة محمد راجح كات	MRN	746 / 2024
Age	66 yrs	ID	001...V...151
Menopausal status	post menopausal	Phone	0100 725 55 20
NRS SCORE		Marital status	married
BMI		Family history	-ve
CO morbidity	HTN	REASON OF MDT	Post - op

Clinical Data

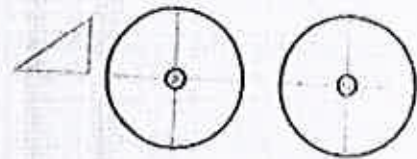
BX: Post wider excision Rt. breast

ER:	PR:	HER 2:	KI67:
+ve	+ve	-ve	10

Sonomammography: IDC GII
LN 0/6
Skin involvement free

Mets Workup:

Wider excision path
free



TNM

Recommendations:

PI/PNo

A

MDT OUTCOME & PLAN

for AD. AZ reports

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Name: Mrs. Fatma Mahmoud Ahmed Ghanem

Date: 4/4/2025

ID:1125003517

Acc.:291229602

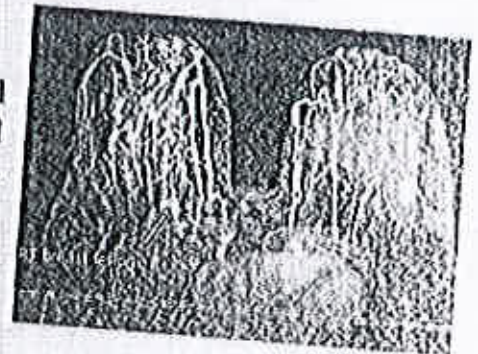
Referring Prof. Dr. Awram Dar Al-Salaam Hospital.

Dynamic MRI Of Both Breasts Using Different Pulse Sequences In Different Planes, Revealed:

Indications: right breast pathological proved invasive ductal carcinoma grade II with fical DCIS (ER +ve, PR +ve & HER2 +1ve)

Findings:

- Heterogeneous fibroglandular tissue
- Mild asymmetric background parenchymal enhancement.
- **Right breast:** upper central posterior third lobulated oval shaped lesion with partially obscured margin, eliciting intermediate signal intensity in T1WI / T2WI, high signal intensity in STIR, in post-contrast dynamic images, heterogeneous moderate mass enhancement (type II kinematic curve) measuring 14 x 13 mm and 11.5 cm distance from the nipple
- **Left breast:**
 - No evidence of spiculated mass lesions or architectural distortion noted.
 - No pathological focus, mass or non-mass enhancement noted.
- Bilateral dilated ducts
- Normal skin thickness. No pathological skin or nipple enhancement.
- No pathological enlarged axillary lymph nodes.
- Intact chest wall.



Conclusion

- **Right breast:** upper central unifocal pathological proved malignancy as described (BIRADS 6)
- **Left breast:** duct ectasia (BIRADS 2)

Dr. Lamiaa Elsharkawy, MD

Prof. Dr. Lamiaa Adel, MD

Female / Breast Imaging & Intervention Consultant



Patient Name : **أ. د. هبة محمد حسن**
Patient No. : **25810071700841**
Path No. : **1815/25**
Sex : **Female**
Age : **66 Y**
Referring Doc :

Request No. : **13005876**
Request Date : **18/03/2025**
Verification Date : **02/06/2025**
Financial Category : **التأمين الصحي**
Department : **Out Patient**
Registered by : **Mrs. Mayada Haggag**

Pathology Report

Clinical data

Acase of invasive duct carcinoma, previous pathology No.1489/25

Nature of specimen

Biopsy

Gross specimen

Two specimens were received:

1. Fibrofatty tissue piece measured 6.5x4x1.5 cm, showed yellowish cut section
2. Fibrofatty tissue measured 4x3x0.5 cm, with rubbery cut section

Microscopic examination

Examination revealed breast tissue showed fibrocystic changes with extensive fat necrosis and inflammation as well as foreign body giant cells reaction

Diagnosis

Right extra-inferior & deep margins :
Free margins. No Malignancy

Signature

Dr. Rana Fatmy
Dr. Heba Hassan

Verified by :
Dr. Heba Hasan

Printing Date: 11/06/2025 11:41:25

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Printing By: 11/06/2025

Patient Name : الأستاذة / فاطمة محمود محمد عليم	Request No. : 13009732
Patient No. : 25810071700841	Request Date : 26/04/2025
Path No. : 1489/25	Verification Date : 07/05/2025
Sex : Female	Financial Category : مريض - مستقرة
Age : 66 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed

Pathology Report

Clinical Data: Right breast cancer, Up-Front
Nature Specimen: Right WLE and SLNs.

Gross Examination:

- * Right SLN: A fibro fatty tissue piece measured 6x4x1 cm entangling 6 nodules up to 2.5x1.5 cm.
- * Right WLE specimen measured 6x4x1 cm, with covering skin ellipse measured 6x1 cm
- * Tumor extent: A firm grayish white firm mass
- * Size: 1.6x1.5 cm
- * Skin involvement: unremarkable grossly
- * Distance from margins: . 0.5cm from deep margin

Microscopic Examination:

- * Right breast, WLE and SLNs biopsy:
- * Histological Type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No
- * Skin involvement: Free
- * Lymph node involvement: No metastatic deposits in lymph nodes 0/6
- * Angioinvasion: No
- * Perineural invasion: No
- * Resection Margins: Inferior & deep margins positive for tumor, free other margins
- * Microcalcifications: Not detected

Diagnosis:

- Right breast, WLE and SLNs Biopsy:
- Invasive duct carcinoma
- Inferior & deep margins positive for tumor, free other margins

Verified by :
Dr. Heba Hasan

Printing Date : 12/05/2025 10:12:02

Page 1 of 2

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CAP

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mizr El Qadima - Cairo, Egypt

Patient Name : الأستاذة / فاطمة محمود محمد عظيم

Patient No. : 25810071700841

Path No. : 1489/25

Sex : Female

Age : 66 Y

Referring Doc :

Request No. : 13009732

Request Date : 26/04/2025

Verification Date : 07/05/2025

Financial Category : مريض - عيادة

Department : Out Patient

Registered by : marwa mohamed youssef

Pathology Report

- No metastatic deposits in lymph nodes 0/6

- Stage: pT1, pN0

Comment : There is disparity between frozen and paraffin results , However the final diagnosis depend on the paraffin results due to error in block quality.

Signature
Dr.Rana Fathy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

Printing Date : 12/05/2025 10:12:02

Page 2 of 2

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CAP

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Micro El Qadima - Cairo, Egypt

Patient Name : الأستاذة / فاطمة محمود محمد عثمان

Patient No. : 25810071700841

Path No. : 642/25

Sex : Female

Age : 66 Y

Referring Doc :

Request No. : 13002166

Request Date : 13/02/2025

Verification Date : 27/02/2025

Financial Category : مريض - مريض

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast malignant looking solid mass.

Nature of Specimen: Tru-cut biopsy, collected at 13/2/2025.

Gross Examination:

- Four grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal DCIS.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Positive.
- * IPT (Positive control showed appropriate reactivity):
- E- Cadherin: Diffuse positive in all tumor cells.
- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- Her2: Weak incomplete membrane reaction in <10% of tumor cells (score 1+).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Right breast malignant looking solid mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: score 1+.
- Ki67: Positive in 10% of tumor cells.

Verified by :

Dr.Toka Medhat

Printing Date : 06.03/2025 10:01:38

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / د. طه محمود محمد غانم	Request No. : 13002166
Patient No. : 25810071700841	Request Date : 13/02/2025
Path No. : 642/25	Verification Date : 27/02/2025
Sex : Female	Financial Category : مائة - مائة
Age : 66 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

* Comment: Although Her2 is Negative, such cases (Score 1+) are predictive of response to Her2-Ab drug conjugates (ADCS) according to latest 2023 ASCO-CAP guidelines.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ghonaim

Dr. Menna Nabil

Dr. Yasmine Essam

Verified by :
Dr. Toka Medhat

Printing Date : 06/03/2025 10:01:38

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: Mrs. Fatma Mahmoud Ahmed Ghanem

Date: 4/2/2025

ID:1125003517

Acc.:291217527

Referring Prof. Dr. Fatma Mahmoud Klama.

Digital 3D soft tissue mammography & tomosynthesis with ultrasound examination of the right breast, revealed:

Indication: Right breast lump

- (ACR 6)
- Right upper central partially lobulated partially obscured dense mass lesion associated with architectural distortion anteriorly.
- Normal breast parenchyma on the left side with no evidences of dense spiculated masses or architecture distortion seen.
- Tomosynthesis clarify the abnormalities.
- No suspicious micro-calcific clusters noted.
- Normal skin thickness and contour of both sides.
- Normal areola / nipple complexes on both sides.

Ultrasonographic examination showed:

- Right upper central irregular heterogenous hypoechoic solid mass lesion with micro-calcific foci within, measures 17 X 14 mm, located at 12 O'clock, deep to skin by 6 mm, surrounded by desmoplastic reaction, directly anterior to muscle but not invade it, (corresponding to palpable lump).
- Dilated retro-areolar ducts bilaterally.
- No left cystic or solid lesions.
- No right cystic lesion.
- Normal skin thickness.
- Non-specific axillary lymph node enlargement with preserved oval shape and central hilum.

Opinion:

- Right upper central probably malignant solid mass lesion (BIRADs 4B).
- Bilateral ductectasia (BIRADs 2).

Recommendation:

- US guided Tru-cut biopsy of right upper central lesion.
- Dynamic MRI breast.

Truly yours;
Dr. Asma' Ahmed, MSc

Spelling checked by Dr. A.A.
Typed by AMB 04/02/25 11:12 AM



Patient Name: Fatma Mahmoud Ahmed Ghanem	Date: 06/03/2025
Exam: C.T CHEST	Age: 66 Year(s)
Medical ID: 746/2025	Pat. ID: 98253

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS:**

- Reported case of right breast cancer.

I Chest:

- Right breast heterogenous parenchyma with UIQ mass lesion measuring about 1.8x 1.6 cm.
- Right prominent axillary lymph nodes are seen.
- Multiple bilateral scattered millimetric pulmonary nodules are seen, the largest about 7 mm seen left lower lobe.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- L5 vertebral body lytic lesion is seen.

Opinion:

- Reported case of right breast cancer revealed:
- Multiple bilateral pulmonary micronodules for follow up.
- L5 vertebral body lytic lesion for bone scan correlation.

Prof. Menna Tassah Hanafy, M.D.

Patient Name: Fatma Mahmoud Ahmed Ghanem	Date : 06/03/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 66 Year(s)
Medical ID: 746/2025	Pat. ID: 98253

ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:

Liver: -

- Average in size (about 16 cm at the right MCL), smooth outline, bright echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent.

Gall Bladder: -

- Average size, normal wall thickness, no stones.
- CBD is not dilated.

Spleen: Average size (About 11 cm), displaying homogenous echopattern.

Kidneys: -

- Are of normal size and site, normal echogenicity, no stone or backpressure changes, no solid or cystic lesion.
- Both ureters are noted dilated.

URINARY BLADDER: Normal filling. Normal wall thickness. no stones or diverticulæ or solid lesion detected.

Uterus: -

- anti-verted, average sized uterus. Endometrial thickness measures about mm, no focal lesions.

Ovaries: - both are average in size with no dominant follicle could be detected.

- No free fluid in the Douglas pouch.
- Para-aortic and pancreatic area: no gross lesion

Opinion:

- Normal study.

Signature Dr. Sherien Ali El Sharkawy
PM 2:08 2025/03/06